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Case Number: 24NNCV05799 Hearing Date: August 3, 2026 Dept: X

#18 - ESTHER ABALOS vs EMBASSY SUITES, LLC

PLAINTIFF'S

MOTION FOR TRIAL PREFERENCE

Plaintiff filed this

personal injury action on November 13, 2024, arising from an October 5, 2024 hotel fall at Embassy Suites in Arcadia.

Plaintiff Esther Abalos moves for trial preference.

Evidentiary Objections

The Lien

Declaration

Defendant filed

evidentiary objections to Dr. Lien's declaration paragraphs 4 through 12 and to Exhibits 1 and 2 on grounds including lack of foundation, lack of personal knowledge, speculation, improper opinion, legal conclusion, hearsay, and lack of authentication.

Paragraph 4 states

that plaintiff's health is in "rapid and unpredictable decline due to cascading, multi-system organ failures," that her physical capacity to participate is "deteriorating rapidly," and that "[a]n Expedited Trial is medically necessary to prevent severe prejudice to her interest in this litigation." The objections are

overruled as to Dr. Lien's medical assessment of plaintiff's condition and sustained as to the final clause regarding litigation prejudice, which is a

legal conclusion.

Paragraph 5

addresses plaintiff's "high mortality risk" and the systemic consequences of her history of ANCA-associated microscopic polyangiitis. The objections are overruled.

Paragraph 6 states

that plaintiff's MPA destroyed her kidneys, that she is dependent on hemodialysis three days a week, and recites specific laboratory findings including a GFR of 7 and creatinine of 6.0. The objections are overruled as to Dr. Lien's medical assessment and sustained as to the recitation of specific laboratory findings from medical records, because unauthenticated medical records are not admissible as independent proof of those facts. (See *Garibay v. Hemmat* (2008) 161 Cal.App.4th 735, 742-743 [where the hospital or medical records are not before the court, an expert medical opinion has no evidentiary basis, and therefore no evidentiary value].)

The objections to

Exhibit 2, and to paragraph 7 to the extent it summarizes the outside article attached as Exhibit 2, are sustained. The medical article in Exhibit 2 is not properly authenticated. The objections to paragraph 7 are overruled as to Dr. Lien's own prognostic opinion and sustained as to the recitation of the study's contents and statistics, which are not admissible through the exhibit itself as independent proof. (Evid. Code, § 1340.)

Paragraph 8

describes the physiological toll of dialysis and plaintiff's cardiac history, including coronary artery disease, pacemaker implantation, atrial fibrillation ablations, and congestive heart failure. The objections are overruled.

Paragraph 9 opines

that plaintiff's physiological reserve is "practically nonexistent" and then recounts details of an October 2024 hospitalization and what plaintiff's treating physicians noted at that time. The objections are overruled as to Dr. Lien's own medical assessment and sustained as to the recitation of hospitalization details and third-party physician statements.

Paragraph 10

recounts a subsequent admission, including hypoxia, hemoptysis, a CT angiogram, a newly discovered 3 cm lung mass with lymphadenopathy, and a hemoglobin value of 8.0, and then gives Dr. Lien's opinion about plaintiff's ability to survive further interventions. The objections are sustained as to the recitation of specific diagnostic findings from records and overruled as to Dr. Lien's medical opinion.

Paragraph 11 states that plaintiff is "in no condition to endure the protracted stress of standard civil litigation" and that delaying trial will "actively prejudice her ability to present her case and participate in her own Trial." The objections are overruled as to Dr. Lien's medical opinion regarding plaintiff's capacity and sustained as to the statement that delay will prejudice her ability to present her case, which is a legal conclusion.

Paragraph 12 refers to plaintiff's advanced age, GFR of 7, end-stage renal failure, cardiac compromise, vasculitis history, hospitalizations, and a newly discovered bleeding 3 cm mass, and concludes that "there is substantial medical doubt Ms. Abalos will survive beyond the next six months." The objections are overruled as to Dr. Lien's ultimate prognosis and sustained to the same limited extent as paragraphs 6, 9, and 10 as to embedded, record-derived factual recitations offered for their truth.

The objections to Exhibit 1, Records from San Gabriel Valley Med Center, are sustained. Dr. Lien identifies Exhibit 1 as a true and correct copy of plaintiff's medical chart that he reviewed in forming his opinions as treating physician. The exhibit is not shown on this record to be authenticated or supported by a custodian/business-records declaration. This ruling does not preclude Dr. Lien from offering medical opinions based on his treatment, examination, or review of records to the extent otherwise admissible; it only excludes Exhibit 1 as independent proof. (Garibay, *supra*, 161 Cal.App.4th at p. 743 ["Physicians can testify as to the basis of their opinion, but this is not intended to be a channel by which testifying physicians can place the opinion of out-of-court physicians before the trier of fact."].)

In all other respects, the objections are overruled. Dr. Lien states adequate foundation for medical opinions based on treatment, clinical evaluation, and record review.

The Norton Declaration

Defendant filed written objections to paragraphs 2, 5, 6, and 7 of the Norton declaration on grounds including lack of foundation, lack of personal knowledge, speculation, improper opinion, legal conclusion, hearsay, and lack of authentication.

The objections to paragraph 2 are sustained.

Paragraph 2 recounts the October 5, 2024 incident, plaintiff's injuries, and subsequent shoulder surgery. Attorney affidavits on a motion for preference may be made on information and belief only as to medical diagnosis and prognosis. (Code Civ. Proc., § 36.5.)

The objections to paragraph 5 are overruled, except the legal-conclusion objection is sustained only as to the clause stating that delay is "directly prejudicing her ability to meaningfully participate in her trial." Paragraph 5 expressly states that it is made on information and belief, and addresses plaintiff's diagnoses, prognosis, and resulting functional limitations.

The objections to paragraph 6 are overruled, except the legal-conclusion objection is sustained as to the phrase that the motion was brought "to prevent prejudicing Ms. Abalos's substantial interest in this action." Paragraph 6 otherwise identifies and quotes Dr. Lien's declaration.

The objections to paragraph 7 are overruled as to the statements that Dr. Lien opined there is substantial medical doubt plaintiff will survive beyond six months and that the trial setting conference is scheduled for March 3, 2027. They are sustained as to the statements that trial "could easily be delayed another one to two (1-2) years," that plaintiff "cannot reasonably be expected to wait as long as two-and-a-half years," and that "[p]reference is therefore necessary and warranted."

Tentative Ruling

Plaintiff's motion for trial preference is GRANTED. The

Court finds that plaintiff Esther Abalos is over 70 years of age, has a substantial interest in this action, and that her health is such that preference is necessary to prevent prejudicing her interest in the litigation.

The Trial Setting

Conference set for March 3, 2027 is advanced and placed off-calendar.

The prior order

requiring the parties to mediate by February 26, 2027 is amended such that mediation must be completed no later than November 1, 2026.

The Court also sets
the following dates:

Final Status Conference: Thursday, November 19, 2026, at 9:00 a.m. in Dept.
X.

Jury Trial:

Tuesday, December 1, 2026, at 9:00 a.m. in Dept. X.

The Court estimates
trial as lasting five court days.

Plaintiff has posted
jury fees.

The Court further
orders the parties to comply with the Court's Trial Preparation Order, which
may be found on the LASC website (www.LACourt.org), "Online Services" section, under the
"Courtroom Information" link for Department X (Alhambra
Courthouse). Trial counsel are ordered to personally appear – in
court – for the Final Status Conference with a copy of their lodged
binders so that same can be reviewed in preparation for trial.

Legal Authority and
Analysis

Code of Civil

Procedure section 36 provides, "A party to a civil action who is over 70 years
of age may petition the court for a preference, which the court shall grant if

the court” finds the party “has a substantial interest in the action as a whole” and the party’s health “is such that a preference is necessary to prevent prejudicing the party’s interest in the litigation.” (Code Civ. Proc., § 36, subd. (a).)

“The standard under subdivision (a), unlike under subdivision (d), which is more specific and more rigorous, includes no requirement of a doctor’s declaration. To the contrary, a motion under subdivision (a) may be supported by nothing more than an attorney’s declaration ‘based upon information and belief as to the medical diagnosis and prognosis of any party.’ [Citations.]” (Fox v. Superior Court (2018) 21 Cal.App.5th 529, 534.) Once a party “meets the requisite standard for calendar preference under subdivision (a), preference must be granted. No weighing of interests is involved.” (Id., at p. 535.) This is so, even against the opposing party’s “interest in having adequate time to prepare for trial.” (Ibid.)

A case that is granted preference must be set for trial “not more than 120 days from that date and there shall be no continuance beyond 120 days from the granting of the motion for preference except for physical disability of a party or a party’s attorney, or upon a showing of good cause stated in the record.” (Code Civ. Proc., § 36, subd. (f).)

Here, Plaintiff was born on May 7, 1946, is now 80 years old, and claims injury in this action. This establishes a substantial interest in the action as a whole. Plaintiff’s counsel asserts that plaintiff suffers from ANCA-associated microscopic polyangiitis, end-stage renal disease with a reported GFR of 7 mL/min/1.73 m², three-times-weekly hemodialysis dependence, coronary artery disease, atrial fibrillation, congestive heart failure, sick sinus syndrome, and a 3-cm lung mass, and that these conditions cause fatigue, shortness of breath, cognitive foggiess, and impaired ability to focus and communicate. (Norton Decl., ¶ 5.) Dr. Thoi H. Lien declares that he is plaintiff’s primary treating physician and that, based on his medical expertise, clinical evaluations, and review of medical records, plaintiff’s health is in rapid and unpredictable decline due to cascading multi-system organ failure. (Lien Decl., ¶¶ 3-4.) Both Plaintiff’s counsel and Dr. Lien describe a declining ability to participate meaningfully in litigation.

Defendant opposes

the motion on the grounds that discovery remains ongoing, plaintiff's roommate may need to be deposed, an independent medical examination may be necessary, defendant is evaluating a possible summary judgment motion, and plaintiff relies on October 2024 records and an outside article rather than updated medical documentation. Defendant argues that plaintiff has not shown a new medical emergency or a material change since October 2024 and faults the motion for relying on older records and an outside article.

That argument does not defeat relief under section 36(a). Section 36(a) does not impose a changed-circumstances requirement, and section 36.5 expressly permits counsel to support a section 36(a) motion by declaration on information and belief as to diagnosis and prognosis. Defendant's concerns do not permit the Court to deny an otherwise qualifying section 36(a) motion. Once the statutory findings are made, the preference is mandatory. (Code Civ. Proc., § 36.)

As Plaintiff has shown both a substantial interest in the action and that her present health is such that trial preference is necessary to prevent prejudice to her interest in the litigation, the motion is granted.